



Abdominal Pain Vomiting and Diarrhea

History

- Age
- Time of last meal
- Last bowel movement/emesis
- Improvement or worsening with food or activity
- Duration of problem
- Other sick contacts
- Past medical history
- Past surgical history
- Medications
- Menstrual history (pregnancy)
- Travel history
- Bloody emesis / diarrhea

Signs and Symptoms

- Pain
- Character of pain (constant, intermittent, sharp, dull, etc.)
- Distention
- Constipation
- Diarrhea
- Anorexia
- Radiation

Associated symptoms:

Fever, headache, blurred vision, weakness, malaise, myalgias, cough, headache, dysuria, mental status changes, rash

Differential

- CNS (increased pressure, headache, stroke, CNS lesions, trauma or hemorrhage, vestibular)
- Myocardial infarction
- Drugs (NSAID's, antibiotics, narcotics, chemotherapy)
- GI or Renal disorders
- Diabetic ketoacidosis
- OB-Gyn disease (ovarian cyst, PID, Pregnancy)
- Infections (pneumonia, influenza)
- Electrolyte abnormalities
- Food or toxin induced
- Medication or Substance abuse
- Psychological

	Consider Blood Glucose Analysis Procedure
B	12 Lead ECG Procedure
	IV or IO Access Protocol UP 6
P	Cardiac Monitor
	Age Appropriate Diabetic Protocol AM 2/ PM 2 <i>if indicated</i>
	Pain Control Protocol UP 11 <i>if indicated</i>
	Age Appropriate Cardiac Protocol(s) <i>if indicated</i>

Serious Signs/ Symptoms
Hypotension, poor
perfusion, shock

YES

NO

A	Normal Saline IV TKO Or Saline Lock
	Ondansetron 4 mg IV / IO / ODT / PO / IM Peds: 0.2 mg/kg Peds Maximum 4 mg May repeat in 15 minutes
	<u>If no response in adults</u> Promethazine 12.5 mg PO / IM / IVPB May repeat x 1 as needed

	IV or IO Access Protocol UP 6 Consider 2 Large Bore sites
A	Normal Saline 500 mL Bolus Repeat as needed Titrates SPB ≥ 90 mmHg Maximum 2 L Peds: 20 mL/kg IV / IO Repeat as needed Titrates to Age Appropriate SBP $\geq 70 + (2 \times \text{Age})$ Maximum 60 mL/kg
	Ondansetron 4 mg IV / IO / ODT / PO / IM Peds: 0.2 mg/kg Peds Maximum 4 mg May repeat in 15 minutes
	Age Appropriate Hypotension/ Shock Protocol AM 5/ PM 3 <i>if indicated</i>

Monitor and Reassess

Notify Destination or
Contact Medical Control

Age Specific Blood Pressure indicating possible shock

Age 0 – 28 days: SBP < 60

Ages ≥ 1 month: SBP < 70

Age 1 – 9: SBP < $70 + (2 \times \text{Age})$

Ages 10 – 64: SBP < 90

Ages ≥ 65 : SBP < 110

All ages Shock Index:
HR > SBP

Promethazine:

FDA advises that deep intramuscular (IM) administration or intravenous piggyback (IVPB) are acceptable under strict adherence to the new guidelines (see pearls)



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Pearls

- **Recommended Exam:** Mental Status, Skin, HEENT, Neck, Heart, Lungs, Abdomen, Back, Extremities, Neuro
- **Abdominal/ back pain in women of childbearing age should be treated as pregnancy related until proven otherwise.**
- **The diagnosis of abdominal aneurysm should be considered with abdominal pain, with or without back and/ or lower extremity pain or diminished pulses, especially in patients over 50 and/ or patients with shock/ poor perfusion. Notify receiving facility early with suspected abdominal aneurysm.**
- **Consider cardiac etiology in patients > 35, diabetics and/ or women, especially with upper abdominal complaints.**
- **Heart Rate:** Tachycardia is one of the first clinical signs of dehydration and volume depletion and typically increases as dehydration becomes more severe.
- **Nausea without vomiting should be treated like vomiting. Patient will benefit from symptom control with antiemetic even if not actively vomiting.**
- **Promethazine (Phenergan):**
May cause sedative effects in pediatric patients and in ages ≥ 65 , and the debilitated, etc.)
When giving promethazine, PO and IM is preferred over IV administration. If giving IV, dilute with 100 mL of normal saline and administer slowly over at least 20 minutes as it can cause severe chemical irritation and tissue damage. Promethazine should be administered through large patent veins, avoid veins in the hand and wrist.
- Isolated vomiting in children is common but can be a sign of more serious pathology. Pyloric stenosis, bowel obstruction, and CNS processes (bleeding, tumors, or increased CSF pressures) all often present with vomiting.
- Vomiting and diarrhea are common symptoms, but can be the symptoms of uncommon and serious pathology such as stroke, CO poisoning, acute MI, new onset diabetes, diabetic ketoacidosis (DKA), and organophosphate poisoning. Maintain a high index of suspicion for serious pathology.